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Guideline for Remuneration of Volunteers in Phase 1 Clinical Trials

This document has been created to guide applicants/sponsors of clinical trials during the use of registered or unregistered medicines in approved clinical trials. It represents the South African Health Products Regulatory Authority's (SAHPRA) current thinking on participant reimbursement during clinical trial participation and proposes a minimum compensation that can be paid. It is not intended as an exclusive approach, and SAHPRA reserves the right to request any additional information and may make amendments in accordance with the current knowledge at the time.

Document History

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2		

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Glossary

Abbreviation/ Term	Meaning
AA	Automobile Association of South Africa
CIOMS	Council for International Organizations of Medical Sciences
CRO	Contract Research Organization
eCOA	Electronic Clinical Outcome Assessments
LMIC	Low- to Middle-Income Country
PI	Principal Investigator
NHREC	National Health Research Ethics Council
REC	Research Ethics Committees
SAHPRA	South African Health Products Regulatory Authority
TIE	Time, Inconvenience, and Expense
WCG	WIRB-Copernicus Group

1. BACKGROUND

According to the National Health Research Ethics Council (NHREC) (2024) and the Council for International Organisations of Medical Sciences (CIOMS) (2016) guidelines, clinical trial participants must be compensated appropriately for their time (T), inconvenience (I), and expenses (E) [1] [2].

The South African Health Products Regulatory Authority (SAHPRA) has a clear policy in place for the remuneration of participants taking part in phase 2-4 trials [3]. However, there is no such policy in place for healthy volunteers participating in phase 1 trials. Most of the Sponsors/Applicants and Pharmaceutical Companies/Contract Research Organizations (CRO) are largely following the same guidelines for these trials.

Phase 1 healthy volunteer trials, in which individuals are exposed to the risks of receiving investigational products (IPs) with no probability for medical benefit, typically offer more substantial payments than other types of clinical research. Additionally, these trials frequently require healthy volunteers to undergo more intense medical monitoring, including possible confinement in a residential research facility for days or weeks. This raises ethical concerns of coercion with unduly inducing participation with offers of high compensation on one hand and exploiting participants with low payments on the other hand [4].

The literature is sparse with regard to the compensation of these volunteers, especially in South Africa. Elliot and Abadie describe how some healthy individuals use phase 1 trials as their sole source of income [5]. While this appears to be primarily a US phenomenon, other countries also report serial phase 1 trial participation among healthy volunteers motivated to earn income through clinical trials. An analysis by Fisher *et al* highlighted inequalities and wide variation in phase 1 compensation. The smallest was a USD150 vaccine study that did not require any clinical confinement, whereas the largest was a USD13,000 oncology study that required healthy volunteers to spend 34 consecutive days and nights in the research facility [4]. According to WCG, compensation for Phase 1 studies usually works out to be about \$10- \$20 an hour. It is noted that there are some Phase 1 clinical studies, e.g. studies involving patients with cancer or another potentially deadly disease, vaccine studies, and studies involving additions to new age groups (such as paediatrics or adolescents), that generally follow the phase 2-4 model because they fall more in the clinical care continuum [6]. Phase 1 in this guideline speaks to mainly healthy volunteers exposed with no probability for medical benefits.

With the increase in Phase 1 studies being conducted in South Africa, the need for separate guidelines on Phase 1 remuneration is essential to ensure fair volunteer compensation. Three plausible models of payment have been discussed – the market model, the wage-payment model, and the reimbursement model [7]. However, in the opinion of Grant and Sugarman, the two former models are potentially

susceptible to undue inducement [8]. Bearing this in mind and given the success of the current guidelines for phase 2-4 trials, it seems feasible to amend these guidelines to be relevant to phase 1 trials conducted in South Africa.

2. PURPOSE AND SCOPE

This document guides Applicant/Sponsor/Investigators conducting Phase 1 studies and applies to Phase 1 studies conducted in South Africa.

The guideline is only applicable to Phase 1 clinical trials, especially First-In-Human studies. Due to the additional risks associated with Gene Transfer Therapy studies, however, this guideline does not apply and participant remuneration in these trials will need to be provided on a case-by-case basis.

3. LEGAL PROVISION

This guideline is established in terms of Regulation 30 of the Medicines and Related Substances Act, 101 of 1965, on the conduct of clinical trials.

4. MODEL FOR REIMBURSEMENT OF CLINICAL STUDY-RELATED VISITS

4.1 Time Spent:

Based on the information provided by WCG and factoring in South Africa being a low- to middle-income country (LMIC) with a more vulnerable population, R100 per hour is recommended. In general, a standard scheduled visit should not take longer than 3 hours. Thus, R300 is recommended for the time component of a standard scheduled visit.

Note:

- 4.1.1** More intensive visits may, however, require additional time at the site. This would necessitate a higher “compensation” as the participant is now spending an extended period of time away, resulting in them missing the majority of their work/school day as opposed to a portion of it.

4.2 Inconvenience (Considered in the light of travel/distance from home to a site):

Based on rates prescribed by the Automobile Association (AA) of South Africa [9], it is recommended that R4.75 per km travelled is paid. The travel cost is broken down into 3 categories (0-25, 26-50, and >50 km). For example, a participant falling in the first category will receive R237.50 travel compensation (R4.75 x 25 km x 2 [return trip]). This should be rounded up to R250 for convenience and to make provision for any parking costs.

Note:

- 4.2.1** It would be ideal for each site PI to decide on the radius around the trial site on which

maximum recruitment efforts would be focused. This would allow for a standardised approach to be adopted per site and per study, leading to less per participant variability of travel-related costs. When participants are identified beyond the predefined catchment, additional compensation could be explored on a case-by-case basis.

4.3 Expenses:

It is recommended that all participants be offered a meal to the value of R100. If this amount is included in the ICF reimbursement total, then the R100 should be removed from the total, documented in the relevant documentation and explained to the participant prior to this adjustment being made. Should the participant be at the site for longer than 3 hours, an additional meal or money should be made available to them.

Table 1: Compensation Model for Standard and Extended Clinic Visits

SAHPRA is of the view that in 2026, a minimum of R650 would be the most appropriate compensation level for a standard Phase 1 participant visit. SAHPRA may review this model periodically, as necessary, taking into account economic conditions and regulatory developments.

Time Distance	Standard Visit 2-3 hours	Extended Visit >3 hours
0-25 km	T = R250 (Return trip [rounded up]) I = R300 (R100 x 3 hours [rounded up]) E = R100 (Meal & beverage) Total = R650	T = R250 (Return trip [rounded up]) I = R300 + R100 per hour (over & above 3 hours) E = R200 (2x meals & beverages) Total = R750 + R100 per hour
26-50 km	T = R500 (Return trip [rounded up]) I = R300 E = R100 (Meal & beverage) Total = R900	T = R500 (Return trip [rounded up]) I = R300 + R100 per hour (over & above 3 hours) E = R200 (2x meals & beverages) Total = R1 000 + R100 per hour
>50 km	T = R500 + R4.75 per km (over & above 50 km) I = R300 E = R100 Total = R900 + R4.75 per km	T = R500 + R4.75 per km (over & above 50 km) I = R300 + R100 per hour (over & above 3 hours) E = R200 (2x meals & beverages) Total = R1 000 + R100 per hour + R4.75 per km

4.4 Additional inconvenience:

Some studies call for more invasive procedures (e.g., PK sampling) or strenuous procedures (e.g., Stress ECGs), which are over and above the standard inconveniences of a study visit. Therefore, a case can be made for additional compensation. Other inconveniences which are not always so apparent need to also be compensated for, such as an HIV trial in which participants are required to abstain from sexual activity and product use for specified periods. These additional procedures must be described in the Protocol and Informed Consent Form, the additional remuneration specified, and justification of these amounts provided. However, if on the visit day the participant reports having not adhered to these procedures, the specified amount should be deducted from the overall ICF amount, documented and explained to the participant.

4.5 Scheduled telephonic visits:

Telephonic visits are generally conducted in one of the following scenarios:

- The protocol specifies that a scheduled visit be conducted telephonically as opposed to onsite, or
- A volunteer is unable to attend the scheduled visit for a specific reason, and the site is instructed by the protocol and/or CRA to conduct the visit telephonically.

This is still considered to be a trial-related activity, and the participant should thus be remunerated for this task. A minimum amount of R250 is recommended.

4.6 Home-based/virtual consultation visits:

When permitted by the protocol, a scheduled visit may be conducted either at home or virtually. The participants must be remunerated for time and inconvenience. A minimum of R300 per visit is recommended; this could be increased depending on the time and complexity of the visit.

4.7 Unscheduled visits:

Unscheduled visits may occur on site, telephonically, virtually, or at home. The same remuneration as detailed above applies.

4.8 Screen Failures:

A screen failure occurs when a potential volunteer is invited to the site, signs an informed consent, and undergoes at least part of a screening visit (such as blood tests, ECG, and/or medical history review), but ultimately does not meet the inclusion/exclusion criteria and therefore does not proceed to randomization or treatment.

- When the screening visit has been concluded in its entirety, the full remuneration is due to the participant.

- When the screen failure is identified before all the screening procedures have been completed and the patient has not necessarily been allocated a screening number, partial reimbursement for the visit is expected. This should be 20-50% of the total reimbursement.

4.9 Parent/Legal guardian/caregiver:

In studies where the participant requires a parent/legal guardian/caregiver to be present, the same remuneration scheme applies for both the participant and the parent/legal guardian/caregiver. However, the inconvenience of large blood draws, additional sampling or lifestyle restriction reimbursement would not be applicable to the parent/legal guardian/caregiver.

4.10 Patient diary completion:

The completion of paper and/or electronic diaries between visits is increasingly being incorporated into clinical trials. This amounts to an additional trial-related activity and can be inconvenient and burdensome for trial participants to complete these diaries. The participant should thus be remunerated for this task. A minimum amount of R100 to R150 per diary entry is recommended, depending on the extent of the trial-related procedures (e.g. dosing, temperature measurements, etc) that need to be captured.

In addition, when using electronic diaries, it must be highlighted that most people in South Africa do not have access to free wi-fi. Free wi-fi and/or sufficient airtime should also be made available to all participants to complete the necessary trial-related activities. Alternatively, sponsors can provide devices that have data preloaded.

4.11 Electronic Clinical Outcome Assessments (eCOA), including the use of tablets, smartphones, wearables, and other such devices:

As for 4.10 above, participants need to be remunerated appropriately for this task. In addition, sufficient airtime needs to be made available to all participants, and/or a device with pre-loaded data need to be provided by the Sponsor.

4.12 In-patients:

Volunteers are frequently required to be confined in a residential research facility as part of their clinical trial participation for a few hours or even days. This can be burdensome and inconvenient to the volunteer and may involve missing work/school and/or incurring additional costs such as babysitting or child-minding.

- 4.12.1** For in-patient stays up to 12 hours, an amount of R150 per hour is recommended. In addition, all volunteers must be provided with standard meals, relevant transport costs, and

compensation for any additional procedures conducted during this period of confinement, such as blood draws, stress ECGs, intravenous infusions, etc.

- 4.12.2** For in-patient stays longer than 12 hours, the volunteer should be reimbursed at an additional amount of R900 per 12-hour block thereafter. In addition, all volunteers must be provided with standard meals, relevant transport costs, and compensation for any additional procedures conducted during this period of confinement, such as blood draws, stress ECGs, intravenous infusions, etc.

Table 2: Compensation Model for In-patients

Length of in-patient stay	12-hour stay	24-hour stay	Longer than 24-hour stay
	T = R150 per hour In addition, all volunteers must be provided with standard meals, relevant transport costs, and compensation for any additional procedures conducted during this period of confinement, such as blood draws, stress ECGs, intravenous infusions, etc. This should be in line with the details outlined above.	T (1st 12 hours) = R150 per hour x 12 = R1 800 T (2nd 12 hours or part thereof) = R900 Total = R2 700 In addition, all volunteers must be provided with standard meals, relevant transport costs, and compensation for any additional procedures conducted during this period of confinement, such as blood draws, stress ECGs, intravenous infusions, etc. This should be in line with the details outlined above.	T (1st 12 hours) = R150 per hour x 12 = R1 800 T (2nd and subsequent 12 hours or part thereof) = R900 per 12-hour block Total = R2 700 + (R900 x number of subsequent 12-hour blocks) For example: 48-hour stay: R2 700 + (2 x R900) = R4 500 72-hour stay: R2 700+ (4 x R900) = R6 300 96-hour stay: R2 700 + (6 x R900) = R8 100 In addition, all volunteers must be provided with standard meals, relevant transport costs, and compensation for any additional procedures conducted during this period of confinement, such as blood draws, stress ECGs, intravenous infusions, etc. This should be in line with the details outlined above.

5. References

- [1] National Department of Health. 2024. South African in Health Research Guidelines: Principles, Processes and Structures.
- [2] Council for International Organizations of Medical Sciences (CIOMS), *International Ethical Guidelines for Health-related Research involving Human Subjects*, 2016
- [3] South African Health Products Regulatory Authority, *Guideline for clinical trial participant time, inconvenience and expense (TIE) compensation model*, South Africa, 2022.
- [4] Fisher JA, McManus L, et al. *Phase I trial compensation: How much do healthy volunteers actually earn from clinical trial enrollment*: Clinical Trials 2021; 18(4): 477-487
- [5] Elliott C and Abadie R. *Exploiting a research underclass in phase I clinical trials*. N Engl J Med 2008; 358: 2316-2317
- [6] WCG, *The IRB's Perspective: Determining Compensation for Clinical Trial Participation*, <https://www.wcgclinical.com/insights/irb-optimal-compensation-clinical-trial-participants/>, 2021
- [7] Dickert N and Grady C. *What's the Price of a Research Subject? Approaches to Payment for Research Participation*. N Engl J Med 1999; 341: 198-203
- [8] Grant RW and Sugarman J. *Ethics in Human Subjects: Do incentives matter?* J Medicine and Philosophy 2004; 29(6): 717-738
- [9] Prescribed rate per kilometer: <https://accountingacademy.co.za/news/read/new-sars-tax-free-rate-per-km-1>

6. VALIDITY

This guideline is valid for a period of 5 years from the effective date of implementation. It will be reviewed on this timeframe or as and when required.